

Acceptability

Qualitative evidence from a diverse range of settings, indicates that while women generally appreciate the interventions and information provided during antenatal visits, they are less likely to engage with services if their beliefs, traditions and socioeconomic circumstances are ignored or overlooked by health-care providers and/or policy-makers (high confidence in the evidence) (22). This may be particularly pertinent for an intervention like acupuncture, which

may be culturally alien and/or poorly understood in certain contexts. In addition, where there are likely to be additional costs associated with treatment or where the treatment may be unavailable (because of resource constraints), women are less likely to engage with health services (high confidence in the evidence).

Feasibility

A lack of resources may limit the offer of treatment for this condition (high confidence in the evidence) (45).

D.5: Interventions for constipation

RECOMMENDATION D.5: Wheat bran or other fibre supplements can be used to relieve constipation in pregnancy if the condition fails to respond to dietary modification, based on a woman's preferences and available options. (Recommended)

Remarks

- Dietary advice to reduce constipation during pregnancy should include promoting adequate intake of water and dietary fibre (found in vegetables, nuts, fruit and whole grains).
- For women with troublesome constipation that is not relieved by dietary modification or fibre supplementation, stakeholders may wish to consider intermittent use of poorly absorbed laxatives.

Summary of evidence and considerations

Effects of interventions for constipation compared with other, no or placebo interventions (EB Table D.5)

The evidence on the effects of various interventions for constipation in pregnancy was derived from a Cochrane review to which only two small RCTs involving 180 women contributed data (161). Both studies were conducted in the United Kingdom among pregnant women with constipation. One compared fibre supplementation with no intervention (40 women), the other compared stimulant laxatives with bulk-forming laxatives (140 women). No perinatal outcomes relevant to this guideline were reported.

Fibre supplementation versus no intervention

Evidence from the small study evaluating fibre supplementation versus no intervention on constipation relief (reported as mean frequency of stools) was assessed as being very uncertain.

Stimulant laxatives versus bulk-forming laxatives

Two stimulant laxatives were used in this 1970s study, senna and Normax®. The latter (containing dantron) is potentially carcinogenic and now only used in terminally ill people; however, data on stimulant

laxatives were not available separately for senna. Evidence on relative symptom relief, side-effects (abdominal discomfort, diarrhoea), and maternal satisfaction for stimulant laxatives versus bulk-forming laxatives (sterculia with or without frangula) was assessed as being very uncertain.

Additional considerations

- Various bulk-forming (wheat bran or oat bran fibre supplements, sterculia, methylcellulose, ispaghula husk), osmotic (lactulose) and stimulant laxatives (senna) are available as over-the-counter medications for constipation and are not known to be harmful in pregnancy (166).
- The absorption of vitamins and mineral supplements could potentially be compromised by laxatives.

Values

See "Women's values" at the beginning of section 3.D: Background (p. 74).

Resources

Costs will vary according to the intervention and region. Cereal fibre supplements can be relatively low-cost at around US\$ 1.5 per 375 g bag of wheat bran.⁶

6 Based on Internet search.

Equity

It is not known whether interventions to relieve constipation might impact inequalities.

Acceptability

Qualitative evidence from a range of LMICs suggests that women may be more likely to turn to traditional healers, herbal remedies or TBAs to treat these symptoms (moderate confidence in the evidence) (22). Evidence from a diverse range of settings indicates that while women generally appreciate

the interventions and information provided during antenatal visits, they are less likely to engage with services if their beliefs, traditions and socioeconomic circumstances are ignored or overlooked by health-care providers and/or policy-makers (high confidence in the evidence).

Feasibility

Other qualitative evidence suggests that a lack of resources may limit the offer of treatment for constipation (high confidence in the evidence) (45).

D.6: Interventions for varicose veins and oedema

RECOMMENDATION D.6: Non-pharmacological options, such as compression stockings, leg elevation and water immersion, can be used for the management of varicose veins and oedema in pregnancy based on a woman's preferences and available options. (Recommended)

Remarks

- Women should be informed that symptoms associated with varicose veins may worsen as pregnancy progresses but that most women will experience some improvement within a few months of giving birth.
- Rest, leg elevation and water immersion are low-cost interventions that are unlikely to be harmful.

Summary of evidence and considerations

Effects of interventions for varicose veins and oedema compared with other, no or placebo interventions (EB Table D.6)

The evidence on the effects of various interventions for varicose veins in pregnancy was derived from a Cochrane review that included seven small trials involving 326 women with varicose veins and/or oedema, and various types of interventions, including rutoside (a phlebotonic drug) versus placebo (two trials), foot massage by a professional masseur for five days versus no intervention (1 trial, 80 women), intermittent external pneumatic compression with a pump versus rest (1 trial, 35 women), standing in water at a temperature between 29°C and 33°C for 20 minutes (water immersion) versus leg elevation (1 trial, 32 women) and reflexology versus rest (1 trial, 55 women) (160). Another trial comparing compression stockings with rest in the left lateral position did not contribute any data. Fetal and neonatal outcomes relevant to the ANC guideline were not reported in these studies.

Pharmacological interventions versus placebo or no intervention

Only one small trial conducted in 1975 (69 women) contributed data. Low-certainty evidence from this

trial suggests that rutoside may reduce symptoms (nocturnal cramps, paraesthesia, tiredness) associated with varicose veins compared with placebo (69 women; RR: 1.89, 95% CI: 1.11–3.22). However, no side-effect data were reported.

Non-pharmacological interventions versus placebo or no intervention

Low-certainty evidence suggests that *reflexology* may reduce oedema symptoms compared with rest only (55 women; RR: 9.09, 95% CI: 1.41–58.54) and that water immersion may reduce oedema symptoms (leg volume) compared with leg elevation (32 women; RR: 0.43, 95% CI: 0.22–0.83). Low-certainty evidence suggests that there may be little or no difference in oedema symptoms (measured as lower leg circumference in centimetres) between foot massage and no intervention (80 women; MD in cm: 0.11 less, 95% CI: 1.02 less to 0.80 more) and between intermittent pneumatic compression and rest (measured as mean leg volume, unit of analysis unclear) (35 women; MD: 258.8 lower, 95% CI: 566.91 lower to 49.31 higher). Only one study (reflexology versus rest) evaluated women's satisfaction, but the evidence is of very low certainty.

Additional considerations

- Compression stockings combined with leg elevation is the most common non-surgical management for varicose veins and oedema; however, the Cochrane review found no evidence on this practice in pregnancy (160). Compression stockings are also widely used to prevent morbidity in non-pregnant people with varicose veins and the evidence for this practice in a related Cochrane review of compression stockings was generally very uncertain (167).

Values

See “Women’s values” at the beginning of section 3.D: Background (p. 74).

Resources

Postural interventions are low-cost interventions. The cost of compression stockings varies but they can cost more than US\$ 15 per pair. Reflexology and professional massage require specialist training, and are, therefore, likely to be more costly.

Equity

It is not known whether interventions to relieve varicose veins and oedema might impact inequalities.

Acceptability

Qualitative evidence from a range of LMICs suggests that women may be more likely to turn to traditional healers, herbal remedies or TBAs to treat these symptoms (moderate confidence in the evidence) (22). In addition, evidence from a diverse range of settings indicates that while women generally appreciate the interventions and information provided during antenatal visits, they are less likely to engage with services if their beliefs, traditions and socioeconomic circumstances are ignored or overlooked by health-care providers and/or policy-makers (high confidence in the evidence). This may be particularly pertinent for an intervention like reflexology, which may be culturally alien and/or poorly understood in certain contexts. Qualitative evidence shows that, where there are likely to be additional costs associated with treatment or where the treatment may be unavailable (because of resource constraints), women are less likely to engage with health services (high confidence in the evidence).

Feasibility

The evidence also suggests that a lack of resources may limit the offer of treatment for varicose veins and oedema (high confidence in the evidence) (45).